Nursing Essentials

Priority: Protect physical safety and emotional dignity together. Assess bleeding, pain, infection, and the family's preferences.

FOCUS 1

Assess loss

- **Bleeding is not the diagnosis:** evaluate pregnancy location, viability, maternal stability, and the clinical history.
- **Threatened loss:** bleeding may occur while a pregnancy remains viable; avoid promising an outcome.
- **Management:** expectant, medication, or procedural care depends on findings, preference, and clinical eligibility.

FOCUS 2

Assess cervix

- **Typical pattern:** painless cervical dilation in the second trimester, sometimes with pressure or discharge changes.
- **Evaluation:** history, cervical findings, and ultrasound guide the plan; exclude labor, infection, and other causes.
- **Care:** selected clients may receive cerclage or other preventive treatment; teach the individualized return precautions.

FOCUS 3

Plan care

- **Confirm through the clinical team:** decreased movement requires assessment; ultrasound establishes absent cardiac activity.
- **Maternal safety:** evaluate bleeding, infection, and coagulation concerns when indicated; discuss the birth and follow-up plan.
- **Investigations:** offer the recommended evaluation and explain choices without assigning blame.

Support and teaching

Make it stick: Acknowledge the loss, use the family's preferred language, and offer support without blame.

Focus	What to know	Nursing application
Offer choices	privacy, support people, memory-making, and time with the baby when desired; never pressure participation.	Offer choices without pressure
Return urgently	heavy bleeding, fainting, severe pain, fever, or other concerning symptoms.	Explain urgent symptoms
Rh prevention	apply current gestational-age-specific Rh immune globulin guidance; do not assume every early loss requires it.	Use current Rh guidance

RED FLAGS · ESCALATE PROMPTLY

Heavy bleeding, syncope, fever, severe pain, poor perfusion, or concern for self-harm requires urgent assessment and support.